NCCP 00203 · clinician draft

Docetaxel 75 mg/m² · every 3 weeks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Docetaxel 75 mg/m² · every 3 weeks · reviewed supportive care

Active medicines: Docetaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any IV replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess G-CSF according to the exact regimen and individual risk.
- Dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated. Missed doses require contact; 20 mg IV replacement is consultant-directed.
- Reconcile the complete steroid plan with the antiemetic plan; do not add duplicate dexamethasone.
- Low emetic risk: separate prevention/rescue medicines according to the national/local plan. Prophylactic G-CSF may be used after individual risk assessment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00203 v7 · p3 · NCCP antiemetic guidance; reconcile with regimen and individual prescription

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required